

JOHNS HOPKINS MEDICINE – NEUROLOGY REFERRAL INTAKE (FAX RECEIVED)

Received: 03/14/2026 09:12 EST

From: Centreville Family Practice / Fairfax ER

Fax quality: fair (some cutoff text)

Page 1 of 11

REFERRAL COVER SHEET

Referring Provider: Anita Desai, MD (PCP)

Practice: Centreville Family Practice

Phone: (703) 555-8821

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Referred To: Neurology (general?)

Reason for Referral: syncope vs seizure, recurrent LOC, unclear etiology

Priority: Routine (though pt had 2 ER visits recently)

Notes:

Pt reports episodes of passing out. ER suggested neuro follow up. Family worried about “something neurological.” Some shaking reported but unclear. Please eval.

Insurance: CareFirst BCBS

Auth #: pending

Attached: notes, labs, imaging, discharge paperwork (some duplication likely)

Page 2 of 11

PATIENT DEMOGRAPHICS

Name: Daniel K. Hargrove

DOB: 08/19/1989

Age: 36

Sex: M

Address: 14722 Clover Hill Ct, Centreville, VA 20120

Phone: (571) 555-2290

Emergency Contact: Lisa Hargrove (spouse) (571) 555-9981

Employer: IT contractor (remote)

Primary Care: Anita Desai, MD

Allergies: NKDA (though one note says “possible penicillin rash as child?” unclear)

Height: 5’10”

Weight: 196 lb (prior note says 210 lb)

REFERRING PROVIDER NOTE (03/10/2026)

Author: A. Desai, MD

Chief complaint: “passing out spells”

HPI:

36 y/o male with hx of HTN, GERD, anxiety presents for follow up after 2 recent ER visits for LOC. Episodes described as sudden loss of consciousness lasting approx 1–2 minutes. Wife reports “he stiffened and jerked a bit” during most recent event but pt recalls feeling lightheaded beforehand. No clear prodrome in earlier episode.

First episode ~2 months ago while standing in kitchen. Pt felt “weird” then collapsed. No tongue biting per pt. No incontinence (though ER note mentions possible urinary leakage?).

Second episode 02/28/26 while seated at desk. Wife reports staring, then unresponsive, then shaking of right arm > left? lasted maybe 30–60 sec. Post-event confusion ~10 min.

Denies chest pain. Occasional palpitations.

ROS:

- dizziness
- headaches (intermittent, mild)
- no focal weakness reported

Assessment:

Recurrent syncope vs possible seizure activity. Given mixed features (prodrome vs convulsive activity), unclear.

Plan:

Refer neurology

Consider EEG

Cardiology referral also placed

Continue lisinopril

PRIOR NOTE #1 (URGENT CARE – 01/12/2026)

Facility: NovaMed Urgent Care

Chief Complaint: fainting episode

HPI:

Pt reports he “passed out” while making coffee this morning. Felt warm, slightly nauseous. Next

thing remembers is waking up on floor. Wife found him. No witnessed seizure activity at that time. No incontinence. No tongue biting.

Vitals: BP 132/84 HR 78

Assessment: likely vasovagal syncope

Plan: hydration, avoid prolonged standing

PMH copied forward:

HTN

GERD

anxiety

“possible migraines?”

Medications:

lisinopril 10 mg daily

omeprazole 20 mg daily

sertraline 50 mg daily

Page 5 of 11

PRIOR NOTE #2 (ER VISIT – 02/28/2026)

Facility: Fairfax ER

Chief Complaint: seizure?

HPI:

36 y/o male brought by spouse after episode at home. Wife states patient became unresponsive, stared ahead, then had rhythmic jerking movements, mostly right upper extremity. Duration ~1 minute. Post-ictal confusion noted.

Pt denies prior seizures but had “fainting spell” last month.

Exam:

Neuro nonfocal after event

Impression:

Possible new onset seizure vs convulsive syncope

Plan:

CT head

labs

discharge with neuro follow up

Discharge instructions: avoid driving until cleared

Page 6 of 11

PRIOR NOTE #3 (ER VISIT – 03/05/2026)

Facility: Fairfax ER

Chief Complaint: recurrent LOC

HPI:

Pt had another episode at home. This time no clear shaking per pt but wife says “his arm twitched.” Pt recalls feeling “off” beforehand.

Conflicting documentation:

Nursing note: “no convulsions observed”

Spouse statement: “looked similar to last time but shorter”

Exam:

alert on arrival

Assessment: syncope vs seizure

Disposition: discharge

Note: pt anxious about diagnosis

Page 7 of 11

LABS (multiple dates combined)

CMP (02/28/26):

Na 139

K 3.8

Cl 104

CO2 24

BUN 15

Cr 0.9

Glucose 102

CBC:

WBC 6.2

Hgb 14.8

Plt 210

TSH: 1.8

Troponin: negative

Lactate (drawn post event?): 2.8 (slightly elevated)

Urine tox: negative

Random prolactin not obtained

Page 8 of 11

IMAGING REPORTS

CT HEAD WITHOUT CONTRAST (02/28/2026)

Indication: seizure vs syncope

Findings:

No acute intracranial hemorrhage

No mass effect

Ventricles normal

Impression: normal CT head

MRI brain: NOT DONE

Chest X-ray (ER): normal

EKG:

NSR, no acute changes

Holter monitor: pending (cardiology)

Page 9 of 11

MEDICATION LIST (compiled from multiple sources, may be outdated)

lisinopril 10 mg daily

omeprazole 20 mg daily

sertraline 50 mg daily

alprazolam 0.25 mg PRN (rare use per pt, but refill noted monthly?)

ibuprofen PRN

vitamin D supplement

melatonin PRN sleep

ondansetron PRN nausea (from ER visit)

Note: no antiseizure meds started

Page 10 of 11

DISCHARGE SUMMARY (ER – 02/28/26)

Diagnosis: possible seizure

Course:

Patient remained stable. CT negative. Labs unremarkable. Episode concerning for seizure given witnessed movements and post-event confusion. However, initial history included prodrome suggestive of syncope.

Instructions:

No driving

Follow up neurology

Return if recurrent episodes

Patient education provided re: seizure precautions (though diagnosis not confirmed)

Page 11 of 11

PROCEDURE NOTE (EKG INTERPRETATION – 03/05/26)

Rate: 72

Rhythm: sinus

Intervals: normal

No ST changes

Comment: no evidence of arrhythmia at time of recording

Additional comments scribbled (partially cut off):

“consider tilt table? vs neuro workup first?? unclear”

END OF FAXED DOCUMENT (some pages slightly skewed, duplicate sections noted in original transmission)